Hip Examination — OSCE Checklist

Deakin MD2 · MM7 · Time: 14 min

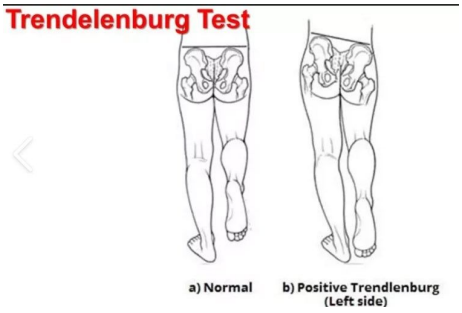
Tick each step as you perform or verbalise it. **Shaded rows (*)** *are optional or done if indicated.* **†** *= added from standard teaching (Talley & O'Connor), not in the Deakin guideline. Order:* **Look → Feel → Move (active, then passive) → Measure → Special tests.** *If fracture suspected: passive movement with great caution, stop if painful or crepitus. Acute loss of circulation or sensation = emergency — don't delay management.*

1. Introduction

☐	Wash hands; introduce self; confirm patient ID; explain and gain consent
☐	Position: start standing
☐	Exposure: underwear ideally (shorts for comfort); shirt lifted to waist; compare with the other side throughout
☐	Ask about pain before touching — "Where is it sore?"
☐	General inspection: well or unwell, in pain? Bedside clues (aids, splints)? Vital signs normal?

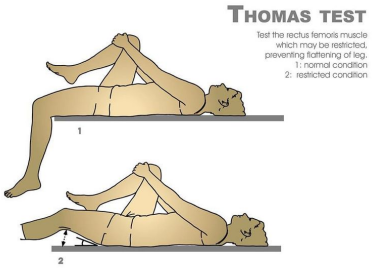
2. Examination

	Step	What to do / say	Abnormal
Look — standing			
☐	Walking in	Walking stick or other aid	
☐	Gait	Normal? Antalgic (short stance on painful side)? Trendelenburg (waddling) †	Hip pain, abductor weakness
☐	Inspect front, back, sides	Deformity, scars/wounds, gluteal and quadriceps wasting, symmetry	Previous surgery, disuse
☐	Trendelenburg test	Stand on one leg (hold your hands †); watch iliac crests; repeat on the other leg	Pelvis drops on the lifted side = weak abductors on the **standing** side



Trendelenburg test

Look — supine			
☐	Re-inspect	Anything missed; leg rotation or shortening †	Fractured NOF: shortened, externally rotated
Feel			
☐	Hip joint	Just distal to midpoint of inguinal ligament; watch the face	Joint tenderness
☐	Greater trochanters	Palpate both	Trochanteric bursitis
Move — supine (active, then passive if pain or restriction)			
☐	Flexion	Straight leg raise (to 90°); knee bent (to 120°)	
☐	Thomas test	Hand under lumbar spine; flex one hip fully until back is flat; watch the other leg	Other leg lifts = fixed flexion deformity
☐	Internal / external rotation	Hip and knee at 90°, rotate foot out (internal) and in (external) †	Internal rotation lost first in OA †
☐	Abduction / adduction	Stabilise the pelvis with your other hand on the opposite ASIS †	
☐	Extension	Patient prone: lift leg off bed	



Thomas test

Measure			
☐	True leg length	ASIS → medial malleolus, both sides	Difference = bone shortening (fracture, congenital, surgery)
☐	Apparent leg length	Umbilicus → medial malleolus, both sides	Difference = pelvic tilt, adduction/abduction, spinal curvature

Special tests (confirm done)			
☐	Trendelenburg, Thomas, leg lengths	Usually completed in Look / Move / Measure	

3. Closing

☐	Thank patient; help them dress; wash hands
☐	Completion: "I would examine the lumbar spine and knee (joints above and below) and do a neurovascular exam of the lower limb †."
☐	Present: "Normal gait with no aids. No deformity, wasting or scars; Trendelenburg negative. No tenderness. Full range of movement; Thomas test negative; true and apparent leg lengths equal."